DESAI METROPOLIS

260000103970311

Name	**: Mr. DEVANSH SURANA**	**VID No.**	: 260000103970311
Age / Gender	: 20 Year(s)/ Male	**PID No.**	: P26426580287178
Contact No.	: +916353063347	**Referred by**	: Dr.SANJAY SHAH
Address	: A220 BHABHAVAN SV NIT COLLEGE KAEVL...	**Registered On**	: 10/10/2026 8:38 AM
Pin code	: 395007	**Collected On**	: 10/10/2026 8:38AM
		Reported On	: 10/10/2026 2:59 PM

Investigation	Observed Value	Unit	Biological Reference Interval
Dengue NS1 Antigen (Serum, Enzyme Immunoassay (EIA))	**Positive,5.67**	Index	Negative: < 0.8 Equivocal: 0.8-1.09 Positive: >= 1.10

Indication:
Dengue NS1 virus antigen usually appears in blood within 24 hours of onset of symptoms to symptoms till 9 days post onset of symptoms.

Interpretation:

Dengue NS1	Dengue IgM	Dengue IgG	Suspected of
+	-	-	Active Dengue Early infection
+	+	-	Primary Dengue infection-
-	+	-	Primary initial stage infection/ cross reactivity
-	+	+	Concurrent/ Recurrent Infection
-	-	+	Past Infection
+	-/+	+	Secondary infection
-	-	-	Not Dengue, other investigations needed.

- If Dengue NS1 is detected positive, concurrent testing with Dengue IgM is advised to know the type of infection-Primary or Secondary.
- Negative NS1 antigen results may occur if the specimen was collected greater than 7 days following symptom onset.

Caution

- A negative Dengue NS1 antigen does not rule out possibility of exposure to or infection with dengue virus.
- False positive Dengue NS1 antigen results may be seen in individuals with other flaviviruses west nile virus as well as Yellow fever.
- High titre of heterophile antibodies or rheumatoid factor may interfere with the results.

Note:
As per regulation, specimen collecting Laboratory is responsible for reporting positive Dengue & Leptospira cases to Municipal corporation.

Associated test:
Dengue IgM Test code (D0004) & Dengue IgG Test code(D0003). Dengue Virus PCR Test code (D0008).

References:

- CDC: Dengue. Accessed 13/09/2019. Available at: www.cdc.gov/Dengue/
- World Health Organization. Dengue: guidance for diagnosis, surveillance and control WHO, Geneva (2009) Available at: http://www.who.int/zoonoses/resources/denguecontrol/en/

Dr. HIRAL SHAH
MD (Pathology)
Consultant Pathologist
Reg No.G- 20971

MEDICAL LABORATORY REPORT

Sample Collected At:
Svnit Surat, Svnit, Ichchhanath, Surat, Gujarat - 395007

Processing Location :
Processing Location Metropolis Healthcare Ltd-Surat1st,6th,7th floor,Maher Park-A,Athwagate,Surat

Name	**: Mr. DEVANSH SURANA**	**VID No.**	: 260000103970311
Age / Gender	: 20 Year(s)/ Male	**PID No.**	: P26426580287178
Contact No.	: +916353063347	**Referred by**	: Dr.SANJAY SHAH
Address	: A220 BHABHAVAN SV NIT COLLEGE KAEVL...	**Registered On**	: 10/10/2026 8:38 AM
Pin code	: 395007	**Collected On**	: 10/10/2026 8:38AM
		Reported On	: 10/10/2026 2:59 PM

Haemogram Advanced Profile - 2

CBC Haemogram

Investigation	Observed Value	Unit	Biological Reference Interval
Erythrocytes			
Haemoglobin (Hb)	14.4	gm/dL	14-18
Erythrocyte (RBC) Count	4.94	mill/cu.mm	4.7-6.0
PCV (Packed Cell Volume)	44.5	%	42-52
MCV (Mean Corpuscular Volume)	90.1	fL	82-101
MCH (Mean Corpuscular Hb)	29.1	pg	27-31
MCHC (Mean Corpuscular Hb Concn.)	32.4	gm/dL	32-36
RDW (Red Cell Distribution Width)	12.1	%	11.5-14.0
Mentzer Index	18		
RBC Morphology			
Remark	Normocytic Normochromic		
Leucocytes			
Total Leucocytes (WBC) Count	**4000**	cells/cu.mm	4300-10300
Absolute Neutrophils Count	2868	cells/cu.mm	2000-7000
Absolute Lymphocyte Count	**640**	cells/cu.mm	1000-3000
Absolute Monocyte Count	460	cells/cu.mm	200-1000
Absolute Eosinophil Count	**0**	cells/cu.mm	20-500
Absolute Basophil Count	20	cells/cu.mm	20-100
Neutrophils	71.7	%	40-80
Lymphocytes	**16.0**	%	20-40
Monocytes	**11.5**	%	2.0-10
Eosinophils	**0.0**	%	1-6
Basophils	0.5	%	0-2
Platelets			
Platelet count	209	10^3/µL	140-440
MPV (Mean Platelet Volume)	**9.9**	fL	6-9.5
Pathologist Remark	Platelet adequate on smear.		

EDTA Whole Blood - Tests done on Automated Five Part Cell Counter. (WBC by Flow Cytometry, RBC & Platelet by sheath flow DC detection method, Hemoglobin by cyanide free sodium lauryl sulphate(SLS) method, HCT by cumulative pulse height detection method, WBC differential count by fluorescence flow cytometry & other parameters calculated). All abnormal hemograms are reviewed & confirmed microscopically.



Dr. CHANDRIKA VANIYA
MD (Pathology)
Consultant Pathologist
Reg No.G-36330

MEDICAL LABORATORY REPORT

Sample Collected At:
Svnit Surat, Svnit, Ichchhanath, Surat, Gujarat - 395007

Processing Location :
Processing Location Metropolis Healthcare Ltd-Surat1st,6th,7th floor,Maher Park-A,Athwagate,Surat

Name	**: Mr. DEVANSH SURANA**	**VID No.**	: 260000103970311
Age / Gender	: 20 Year(s)/ Male	**PID No.**	: P26426580287178
Contact No.	: +916353063347	**Referred by**	: Dr.SANJAY SHAH
Address	: A220 BHABHAVAN SV NIT COLLEGE KAEVL...	**Registered On**	: 10/10/2026 8:38 AM
Pin code	: 395007	**Collected On**	: 10/10/2026 8:38AM
		Reported On	: 10/10/2026 2:59 PM

Investigation	Observed Value	Unit	Biological Reference Interval

Mentzer Index is an automated cell-counter-based calculated parameter (MCV/RBC) used to differentiate cases of Iron Deficiency Anemia (IDA) (MI > 13) from Beta Thalassemia Trait (MI < 13) in patients presenting with microcytic anemia. The Mentzer Index (MI) serves as a screening tool and is not diagnostic on its own.

Advice: If MI is < 13, it is recommended to perform abnormal hemoglobin studies and HbA2 estimation by HPLC (Test Code: A0001) to evaluate for Beta Thalassemia Trait. Iron studies (Test Code: I0286) may also be considered to rule out concurrent iron deficiency. Clinical correlation and further genetic testing are essential for definitive diagnosis.

Malaria Parasite Detection by Smear Examination

(EDTA Whole Blood)

Malaria Parasite examination on thick Smear	Not Detected	Not Detected
Malaria Parasite examination on thin Smear	Not Detected	Not Detected

Method

: Microscopy .

Interpretation:

1. Parasites are usually seen during febrile episode.
2. Peripheral smear has sensitivity of 86.79% and hence repeated smear examination can be required to rule out false negativity.
3. Peripheral smear examination is a screening test and other methods like QBC (quantitative buffy coat), malaria antigen test and PCR should be used for confirmation especially in low parasitic index.
4. Parasitic index reflects severity of infestation (parasites per 100 RBC).
5. Parasitemia after adequate therapy is indicative of resistant strains.

Associated Test:

- "Fever Panel by Multiplex PCR" for early diagnosis of Dengue virus, Chikungunya virus, Salmonella spp., West Nile virus, Plasmodium spp., Rickettsia spp. and Leptospira spp.

-- End of Report --

NABLM0626GJ08114

Test Marked with NABL symbol are in the scope of accrediation

Chandrika

Dr. CHANDRIKA VANIYA
MD (Pathology)
Consultant Pathologist
Reg No.G-36330

Sample Collected At:
Svnit Surat, Svnit, Ichchhanath, Surat, Gujarat - 395007

Processing Location :
Processing Location Metropolis Healthcare Ltd-Surat1st,6th,7th floor,Maher Park-A,Athwagate,Surat